A/PROF CHAMARA BASNAYAKE
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PN 421926PK

FOCUS GASTROENTEROLOGY

25 May 2026

Dr Dinusha Arachchige
Bendigo Medical

Dear Dr Arachchige,

Re: Ms Bron Wyn
DOB: 09/05/1966 Phone: 0458124212
Address: Unit 4/81A Maple Street, GOLDEN SQUARE, VIC, 3555
Email: bronwyn.wyn@education.vic.gov.au

Summary

- **Cessation of domperidone trial due to inadequate symptom control.**
- **Introduction of mirtazapine for its neuromodulatory effects on the oesophagus and stomach.**
- **Conservative management of suspected retrograde cricopharyngeal dysfunction.**
- **No action required by you.**

I had the pleasure of reviewing Bron wyn via telehealth. As we previously discussed, her recent CT scan was clear. We focused today's review on her response to the trial of domperidone and the management of her ongoing symptoms of chest and upper abdominal discomfort.

Bronwyn noted that while taking the domperidone, she was able to burp occasionally and randomly, perhaps once a week. However, the medication has not significantly altered the frequency of her vomiting or the severity of her discomfort. Given that the clinical value appears minimal, we have agreed to cease the domperidone.

I suspect Bronwyn has a degree of retrograde cricopharyngeal dysfunction (RCPD), also known as "inability to burp syndrome." She describes the characteristic gurgling sounds in the lower chest, though her frequent vomiting is somewhat atypical for the classic presentation of this condition. I have specifically discussed the possibility of Botox injections into the upper oesophageal sphincter, which is the standard treatment for RCPD. However, I have decided against this pathway as the most common side effect is increased regurgitation. In Bronwyn's case, I am concerned this would exacerbate her vomiting and lead to a poor clinical outcome.

Impression and Plan

We will now pivot to a trial of mirtazapine. I have explained to Bronwyn that while this is historically classified as an antidepressant, I am using it here as a neuromodulator to modify the nerve pathways of the oesophagus and stomach. It can be quite effective in improving gastric emptying and visceral sensation. I have cautioned her regarding the potential side effects of sedation and increased appetite, which may lead to weight gain.

Bronwyn will commence a low dose of mirtazapine at the end of June. She is to continue her escitalopram 5 mg for the time being, as I am keen to avoid the simultaneous withdrawal of multiple medications. I have also

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provided her with a repeat prescription for esomeprazole 20 mg. I have requested a review in September, approximately eight weeks after she commences the mirtazapine, to evaluate her response.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: A/Prof Michael Hii

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